BSC_EVO_1756	Sacroiliac Joint Injections		
Original Policy Date:	January 1, 2017	**Effective Date:**	July 1, 2025
Section:	6.0 Radiology	**Page:**	Page 1 of 12

Policy Statement

Indications for Sacroiliac Joint Injections (Intraarticular or ligamentous injections only)
Sacroiliac Joint Pain [1,2,3,4]
For the treatment of Sacroiliac Joint (SIJ) pain **ALL** of the following must be met:

- Primarily axial low back pain (below level of L5) which may radiate to the groin or lower extremity
- Pain causing functional disability or average pain level of ≥ 6 on a scale of 0 to 10 related to the requested spinal region.
- A cluster of any three (3) of the following positive provocation exam findings to suggest the diagnosis [5,6,7]:
  - Pelvic (SI) distraction test
  - Pelvic (SI) compression test
  - Sacral Thrust test
  - FABER (Patrick's test)
  - Posterior shear test
  - Yeoman's test
  - Gaenslen's test
  - Thigh Thrust test
- Duration of pain of at least **3 months**
- Failure to respond to non-operative conservative treatment targeting the requested spinal region for a minimum of 6 weeks in the last 6 months unless the medical reason this treatment cannot be done is clearly documented;
  - **OR** details of active engagement in ongoing non-operative conservative treatment if the individual has had prior spinal injections in the same region

Spondyloarthropathy [8]
ALL of the following must be met:

- The individual has experienced ≥ 3 months of low back pain
- Age of onset < 45 years
- Comprehensive pain management program is in place including physical therapy, home exercise, patient education, psychosocial support, and/or oral medication
- Prior history of evidence of sacroiliitis on imaging (i.e., active inflammation on magnetic resonance imaging [MRI] or definite radiographic sacroiliitis grade 2-4 bilaterally *or* grade 3-4 unilaterally)
- **1 or more** spondyloarthropathy features:
  - Inflammatory back pain evidence with **at least 4** of the following criteria present [9]:
    - Age at onset < 40 years
    - Insidious onset
    - Improvement with exercise
    - No improvement with rest
    - Pain at night (with improvement upon getting up)
  - Arthritis

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- Enthesitis of the heel (irritability of muscles, tendons, or ligaments where they enter the bone)
- Uveitis (inflammation of the uvea, the middle layer of the eye)
- Dactylitis (inflammation of a finger or toe)
- Psoriasis
- Crohn's/colitis
- Good response to NSAIDs
- Family history of spondyloarthropathy
- Positive testing for HLA-B27
- Elevated C-reactive protein (CRP)

Imaging Guidance [3,4]

- The sacroiliac joint is commonly identified under image guidance by Fluoroscopy or Computed tomography (CT). CT is less effective than Fluoroscopy regarding observing of the escape of the injectate to the adjacent structures and cannot rule out concurrent intravascular flow. With proper use by skilled interventional pain physicians with ultrasound experience, the use of ultrasound guidance is similar to CT or Fluoroscopy but can have a lower accuracy of needle placement.
- Ultrasound guidance can be an effective alternative if fluoroscopy or CT guided techniques are contraindicated or when radiation exposure is problematic; however, individual patient factors such as poor visualization due to deeper tissue layers (e.g., increased Body Mass Index (BMI) may contribute to substandard image resolution).

NOTE: ALL procedures must be performed under imaging guidance

Diagnostic Purposes for Surgical Planning [3,6]
For diagnostic purposes, **ALL** the following must be met:

- The sacroiliac joint injection is an image-guided, contrast-enhanced intra-articular injection
- At least 75% pain relief for the expected duration of the anesthetic after each diagnostic injection
- After the diagnostic relief period, the individual continues to have pain causing functional disability or average pain level ≥ 6 on a scale of 0 to 10 related to the requested spinal region.
- No more than two diagnostic injections per diagnostic phase
- Documentation of a pre-operative evaluation and plan for SIJ surgery

Repeat Injections [1,3,6]
Sacroiliac joint injections may be repeated only as **medically necessary**. **Each** sacroiliac joint injection requires an authorization, and the following criteria must be met for repeat injections:
Initial Treatment Phase

- Up to 2 sacroiliac joint injections may be performed in the initial treatment phase, no sooner than 2 weeks apart, provided that at least 50% pain relief or significant documented functional improvement is obtained

Therapeutic Phase

- Sacroiliac joint injections may only be repeated after the initial treatment phase if the individual has had at least 50% pain relief or significant documented functional improvement for a **minimum of 2 months** before each therapeutic injection
- The individual continues to have pain causing functional disability or average pain level ≥ 6 on a scale of 0 to 10 related to the requested spinal region.
- The individual is engaged in ongoing active conservative treatment unless the medical reason this treatment cannot be done is clearly documented
- For individuals that have received other interventional pain injections in the lumbar/sacral region (e.g., epidural steroid injection or facet joint injection) since the last SIJ injection, at

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least one repeat positive provocative exam finding is required (pelvic (SI) distraction test, pelvic (SI) compression test, sacral thrust test, FABER (Patrick's test), posterior shear test, Yeoman's test, Gaenslen's test, or thigh thrust).
- A maximum of 4 sacroiliac joint injections may be performed in a 12-month period per region in the therapeutic phase

EXCLUSIONS
These requests are excluded from consideration under this guideline:
- Sacral lateral branch blocks (S1, S2, S3)
- Radiofrequency denervation of the sacroiliac joint

CONTRAINDICATIONS [2,3,4]
- Absolute contraindications
  - Active systemic or spinal infection
  - Skin infection at the site of needle puncture
  - Local malignancy
  - Septic joint
- Relative contraindications
  - Coagulopathy
  - Pregnancy
  - Uncontrollable Diabetes
  - Current and uninterrupted use of blood-thinning medication

NOTE: Refer to Appendix A to see the policy statement changes (if any) from the previous version.

Policy Guidelines

Home Exercise Program (HEP) [13]
The following two elements are required to meet conservative therapy guidelines for HEP:
- Documentation of an exercise prescription/plan provided by a physician, physical therapist, or chiropractor

AND
- Follow-up documentation regarding completion of HEP after the required 6-week timeframe or inability to complete HEP due to a documented medical reason (e.g., increased pain or inability to physically perform exercises).

General Information
It is an expectation that all patients receive care/services from a licensed clinician. All appropriate supporting documentation, including recent pertinent office visit notes, laboratory data, and results of any special testing must be provided. If applicable: All prior relevant imaging results and the reason that alternative imaging cannot be performed must be included in the documentation submitted.

Coding
See the Codes table for details.

Description

Low back pain originating from the SIJ can result from inflammatory conditions such as sacroiliitis, spondyloarthropathy (e.g., ankylosing spondylitis, rheumatoid spondylitis), or from postsurgical or traumatic injury, degeneration (wear and tear), or pregnancy. SIJ pain most often occurs in the buttocks and lower back and may radiate down through the buttocks and the leg. Physical

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examination and radiographic techniques may confirm a diagnosis related to spondyloarthropathy. Physical examination, including provocative maneuvers to elicit pain response, and controlled SIJ injections can help diagnose noninflammatory pain arising from the SIJ.

Risks associated with SIJ dysfunction [3,4]:

- Gait abnormalities
- Scoliosis
- Leg-length discrepancies
- Inflammatory spondyloarthropathies, including ankylosing spondylitis
- Previous spine surgeries
- Connective tissue disorders (e.g., Ehlers–Danlos syndrome)
- Pregnancy associated with ligamentous laxity and hypermobility
- Obesity

Spinal injections for the treatment of SIJ pain syndrome are typically performed as one part of a comprehensive treatment program, but initial treatment usually includes over-the-counter analgesics, home exercise program to improve or maintain spinal mobility, and therapy sessions with a physical therapist involving range-of-motion, stretching, and strengthening exercises.

Sacroiliac joint injections are typically used for the following conditions:

- **Sacroiliac joint (SIJ) syndrome** may be caused by various events, including pain secondary to postsurgical or traumatic injury, degeneration (wear and tear), or pregnancy.
- **Diagnostic SIJ injections** are used to determine if the SIJ pain originates with the SIJ. Diagnostic blocks can reveal (or fail to reveal) that the source of pain is originating from the SIJ; appropriate treatment plan can be developed.
- **Therapeutic SIJ injections** used to treat SIJ pain once it has been determined that the SIJ is the origin of the pain. A therapeutic injection typically includes a corticosteroid and a local anesthetic that can be injected directly into the joint (intra-articular) or into the tissues surrounding the joint (periarticular).
- **Spondyloarthropathy** (also known as spondyloarthritis) is the name for a family of rheumatic diseases that cause arthritis. Sacroiliitis is a key indicator of spondyloarthritis and is diagnosed with imaging. Individuals with spondyloarthropathy are generally managed by rheumatologists.

Related Policies

- N/A

Benefit Application

Benefit determinations should be based in all cases on the applicable member health services contract language. To the extent there are conflicts between this Medical Policy and the member health services contract language, the contract language will control. Please refer to the member's contract benefits in effect at the time of service to determine coverage or non-coverage of these services as it applies to an individual member.

Some state or federal law may prohibit health plans from denying FDA-approved Healthcare Services as investigational or experimental. In these instances, Blue Shield of California may be obligated to determine if these FDA-approved Healthcare Services are Medically Necessary.

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Regulatory Status

- N/A

Rationale

Background

The indications for coverage for the treatment of spondyloarthropathy have been established through criteria developed by the Assessment of SpondyloArthritis International Society (ASAS) for the classification of axial spondyloarthritis. [11] They are in keeping with the benefit guidelines developed by the Centers for Medicare & Medicaid Services (CMS). [12]

Telehealth visits have become routine in modern medical practice. However, sacroiliac joint injections cannot be performed via telehealth encounters. Individuals who can schedule an in- person encounter for injection are expected to also schedule an in-person encounter for provocative physical examination, prior to injection, in order to document the medical necessity of the joint injection.

Medical Necessity

It is generally considered not medically necessary to perform multiple interventional pain procedures on the same date of service. Documentation of a medical reason to perform injections in different regions on the same day can be provided and will be considered on a case-by-case basis (e.g., holding anticoagulation therapy on two separate dates creates undue risk for the patient).

Special Note

Any injection performed at least two years from prior injections in the same region will be considered a new episode of care and the **INITIAL** injection requirements must be met for approval. Events such as surgery on the same spinal region or any new pathology would also prompt a new episode of care.

References

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2. Wu L, Tafti D, Varacallo M. Sacroiliac Joint Injection. StatPearls [Internet]. 2023; https://www.ncbi.nlm.nih.gov/books/NBK513245/.
3. Sayed D, Deer T R, Tieppo Francio V, Lam C M, Sochacki K et al. American Society of Pain and Neuroscience Best Practice (ASPN) Guideline for the Treatment of Sacroiliac Disorders. Journal of pain research. 2024; 17: 1601-1638.
4. Sayed D, Grider J, Strand N, Hagedorn J M, Falowski S et al. The American Society of Pain and Neuroscience (ASPN) Evidence-Based Clinical Guideline of Interventional Treatments for Low Back Pain. Journal of pain research. 2022; 15: 3729-3832.
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https://www.nice.org.uk/guidance/ng65/resources/spondyloarthritis-in-over-16s-diagnosis-and-management-pdf-1837575441349.
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10. Washington State Health Care Authority. Health Technology Clinical Committee Coverage Topic 20160318B – Spinal Injections. 2016; https://www.hca.wa.gov/assets/program/spinal_injections-rr_final_findings_decision_060216.pdf.
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12. Centers for Medicare & Medicaid Services. Sacroiliac Joint Injections and Procedures. 2024; Accessed: 30-Aug-2024. https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleid=59246&ver=17&keyword=sacroiliac&keywordType=starts&areaId=all&docType=NCA,CAL,NCD,MEDCAC,TA,MCD,6,3,5,1,F,P&contractOption=all&sortBy=relevance&bc=1.
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Documentation for Clinical Review

Please provide the following documentation:

- History and physical and/or consultation notes including:
  - Procedure performed and reason for procedure
  - Previous treatment and response (including duration of treatment)
- Radiology report(s), if applicable
- Prior procedure report(s), if applicable

Post Service (in addition to the above, please include the following):

- Procedure report(s)

Coding

The list of codes in this Medical Policy is intended as a general reference and may not cover all codes. Inclusion or exclusion of a code(s) does not constitute or imply member coverage or provider reimbursement policy.

Type	Code	Description
CPT®	27096	Injection procedure for sacroiliac joint, anesthetic/steroid, with image guidance (fluoroscopy or CT) including arthrography when performed
HCPCS	G0260	Injection procedure for sacroiliac joint; provision of anesthetic, steroid and/or other therapeutic agent, with or without arthrography

Policy History

This section provides a chronological history of the activities, updates and changes that have occurred with this Medical Policy.

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Effective Date	Action
01/01/2017	Adoption of National Imaging Associates (NIA) Clinical Guidelines
07/01/2018	NIA Clinical Guideline update
07/01/2019	NIA Clinical Guideline update
07/01/2020	Annual NIA clinical guideline update.
03/01/2021	Annual NIA clinical guideline update. Policy title changed from Sacroiliac Joint Injections to current one.
01/01/2022	Annual NIA clinical guideline update.
01/01/2023	Annual NIA clinical guideline update. Policy title changed from Sacroiliac Joint Injections (with image guidance [fluoroscopy or CT]) to current one.
01/01/2024	Annual NIA clinical guideline update.
07/01/2024	Semi-annual NIA clinical guideline update. Coding update.
07/01/2025	Annual Evolent clinical guideline update.

Feedback

Blue Shield of California is interested in receiving feedback relative to developing, adopting, and reviewing criteria for medical policy. Any licensed practitioner who is contracted with Blue Shield of California or Blue Shield of California Promise Health Plan is welcome to provide comments, suggestions, or concerns. Our internal policy committees will receive and take your comments into consideration. Our medical policies are available to view or download at www.blueshieldca.com/provider.

For medical policy feedback, please send comments to: MedPolicy@blueshieldca.com

Questions regarding the applicability of this policy should be directed to the Prior Authorization Department at (800) 541-6652, or the Transplant Case Management Department at (800) 637-2066 ext. 3507708 or visit the provider portal at www.blueshieldca.com/provider.

Disclaimer: Blue Shield of California may consider published peer-reviewed scientific literature, national guidelines, and local standards of practice in developing its medical policy. Federal and state law, as well as member health services contract language, including definitions and specific contract provisions/exclusions, take precedence over medical policy and must be considered first in determining covered services. Member health services contracts may differ in their benefits. Blue Shield reserves the right to review and update policies as appropriate.

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Appendix A

POLICY STATEMENT	
BEFORE Red font: Verbiage removed	**AFTER** Blue font: Verbiage Changes/Additions
Sacroiliac Joint Injections BSC_NIA_CG_305 **Policy Statement:** **INDICATIONS [1, 2, 3]** **SACROILIAC JOINT (SIJ) INJECTIONS (Intraarticular or ligamentous injections only)** For the treatment of Sacroiliac Joint (SIJ) pain **ALL** of the following must be met: • Primarily axial low back pain (below level of L5) which may radiate to the groin or lower extremity • Pain causing functional disability or average pain level of ≥ 6 on a scale of 0 to 10 related to the requested spinal region. • A cluster of any three (3) of the following positive provocation exam [2, 4, 5] findings to suggest the diagnosis: o Pelvic (SI) distraction test o Pelvic (SI) compression test o Sacral Thrust test o FABER (Patrick's test) o Posterior shear test o Yeoman's test o Gaenslen's test o Thigh Thrust test • Duration of pain of at least **3 months** • Failure to respond to non-operative conservative therapy* targeting the requested spinal region for a minimum of 6 weeks in the last 6 months unless the medical reason this treatment cannot be done is clearly documented; **OR** details of active engagement in ongoing non-operative conservative non-operative therapy* if the individual has had prior spinal injections in the same region **SPONDYLOARTHROPATHY TREATMENT [4, 6, 7]**	**Sacroiliac Joint Injections BSC_EVO_1756** **Policy Statement:** **Indications for Sacroiliac Joint Injections (Intraarticular or ligamentous injections only)** **Sacroiliac Joint Pain** [1,2,3,4] For the treatment of Sacroiliac Joint (SIJ) pain **ALL** of the following must be met: • Primarily axial low back pain (below level of L5) which may radiate to the groin or lower extremity • Pain causing functional disability or average pain level of ≥ 6 on a scale of 0 to 10 related to the requested spinal region. • A cluster of any three (3) of the following positive provocation exam findings to suggest the diagnosis [5,6,7]: o Pelvic (SI) distraction test o Pelvic (SI) compression test o Sacral Thrust test o FABER (Patrick's test) o Posterior shear test o Yeoman's test o Gaenslen's test o Thigh Thrust test • Duration of pain of at least **3 months** • Failure to respond to non-operative conservative treatment targeting the requested spinal region for a minimum of 6 weeks in the last 6 months unless the medical reason this treatment cannot be done is clearly documented; o **OR** details of active engagement in ongoing non-operative conservative treatment if the individual has had prior spinal injections in the same region **Spondyloarthropathy** [8]

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ALL of the following must be met: • The individual has experienced ≥ 3 months of low back pain • Age of onset < 45 years • Comprehensive pain management program is in place including physical therapy, home exercise, patient education, psychosocial support, and/or oral medication • Prior history of evidence of sacroiliitis on imaging (i.e., active inflammation on magnetic resonance imaging [MRI] or definite radiographic sacroiliitis grade > 2 bilaterally *or* grade 3-4 unilaterally) • ***1 or more*** spondyloarthropathy features: ○ Inflammatory back pain with **at least 4** of the following criteria present: ▪ Age at onset < 45 years ▪ Insidious onset ▪ Improvement with exercise ▪ No improvement with rest ▪ Pain at night (with improvement upon getting up) ○ Arthritis ○ Enthesitis of the heel (irritability of muscles, tendons, or ligaments where they enter the bone) ○ Uveitis (inflammation of the uvea, the middle layer of the eye) ○ Dactylitis (inflammation of a finger or toe) ○ Psoriasis ○ Crohn's/colitis ○ Good response to NSAIDs ○ Family history of spondyloarthropathy ○ Positive testing for HLA-B27 ○ Elevated C-reactive protein (CRP) **IMAGING GUIDANCE [1, 2, 3, 8]** The sacroiliac joint is commonly identified under image guidance by Fluoroscopy or Computed tomography (CT). CT is less effective than Fluoroscopy regarding observing of the escape of the injectate to the	ALL of the following must be met: • The individual has experienced ≥ 3 months of low back pain • Age of onset < 45 years • Comprehensive pain management program is in place including physical therapy, home exercise, patient education, psychosocial support, and/or oral medication • Prior history of evidence of sacroiliitis on imaging (i.e., active inflammation on magnetic resonance imaging [MRI] or definite radiographic sacroiliitis grade 2-4 bilaterally *or* grade 3-4 unilaterally) • **1 or more** spondyloarthropathy features: ○ Inflammatory back pain evidence with **at least 4** of the following criteria present[9]: ▪ Age at onset < 40 years ▪ Insidious onset ▪ Improvement with exercise ▪ No improvement with rest ▪ Pain at night (with improvement upon getting up) ○ Arthritis ○ Enthesitis of the heel (irritability of muscles, tendons, or ligaments where they enter the bone) ○ Uveitis (inflammation of the uvea, the middle layer of the eye) ○ Dactylitis (inflammation of a finger or toe) ○ Psoriasis ○ Crohn's/colitis ○ Good response to NSAIDs ○ Family history of spondyloarthropathy ○ Positive testing for HLA-B27 ○ Elevated C-reactive protein (CRP) **Imaging Guidance [3,4]** • The sacroiliac joint is commonly identified under image guidance by Fluoroscopy or Computed tomography (CT). CT is less effective than Fluoroscopy regarding observing of the escape of the injectate to

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adjacent structures and cannot rule out concurrent intravascular flow. With proper use by skilled interventional pain physicians with ultrasound experience, the use of ultrasound guidance is similar to CT or Fluoroscopy but can have a lower accuracy of needle placement. Ultrasound guidance can be an effective alternative if fluoroscopy or CT guided techniques are contraindicated; however, individual patient factors such as poor visualization due to deeper tissue layers (e.g., increased Body Mass Index (BMI) may contributes to substandard image resolution). **NOTE**: ALL procedures must be performed under imaging guidance **DIAGNOSTIC PURPOSES FOR SURGICAL PLANNING [4, 9]** • For diagnostic purposes all of the following must be met: o The sacroiliac joint injection is an image-guided, contrast-enhanced intra-articular injection o At least 75% pain relief for the expected duration of the anesthetic after each diagnostic injection o After the diagnostic relief period, the individual continues to have pain causing functional disability or average pain level ≥ 6 on a scale of 0 to 10 related to the requested spinal region. o No more than two diagnostic injections per diagnostic phase o Documentation of a pre-operative evaluation and plan for SIJ surgery **REPEAT INJECTIONS [2, 4]** Sacroiliac joint injections may be repeated only as medically necessary. **Each** sacroiliac joint injection requires an authorization, and the following criteria must be met for repeat injections: • Up to 2 sacroiliac joint injections may be performed in the initial treatment phase, no sooner than 2 weeks apart, provided that at	the adjacent structures and cannot rule out concurrent intravascular flow. With proper use by skilled interventional pain physicians with ultrasound experience, the use of ultrasound guidance is similar to CT or Fluoroscopy but can have a lower accuracy of needle placement. • Ultrasound guidance can be an effective alternative if fluoroscopy or CT guided techniques are contraindicated or when radiation exposure is problematic; however, individual patient factors such as poor visualization due to deeper tissue layers (e.g., increased Body Mass Index (BMI) may contribute to substandard image resolution). **NOTE**: ALL procedures must be performed under imaging guidance **Diagnostic Purposes for Surgical Planning** [3,6] For diagnostic purposes, **ALL** the following must be met: • The sacroiliac joint injection is an image-guided, contrast-enhanced intra-articular injection • At least 75% pain relief for the expected duration of the anesthetic after each diagnostic injection • After the diagnostic relief period, the individual continues to have pain causing functional disability or average pain level ≥ 6 on a scale of 0 to 10 related to the requested spinal region. • No more than two diagnostic injections per diagnostic phase • Documentation of a pre-operative evaluation and plan for SIJ surgery **Repeat Injections** [1,3,6] Sacroiliac joint injections may be repeated only as **medically necessary**. **Each** sacroiliac joint injection requires an authorization, and the following criteria must be met for repeat injections: ***Initial Treatment Phase*** • Up to 2 sacroiliac joint injections may be performed in the initial treatment phase, no sooner than 2 weeks apart, provided that at

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least 50% pain relief or significant documented functional improvement is obtained • Sacroiliac joint injections may only be repeated after the initial treatment phase if the individual has had at least 50% pain relief or significant documented functional improvement for a **minimum of 2 months** after each therapeutic injection • The individual continues to have pain causing functional disability or average pain level ≥ 6 on a scale of 0 to 10 related to the requested spinal region. • The individual is engaged in ongoing active conservative therapy*, unless the medical reason this treatment cannot be done is clearly documented • For individuals that have received other interventional pain injections in the lumbar/sacral region (e.g., epidural steroid injection or facet joint injection) since the last SIJ injection, at least one repeat positive provocative exam finding is required (pelvic (SI) distraction test, pelvic (SI) compression test, sacral thrust test, FABER (Patrick's test), posterior shear test, Yeoman's test, Gaenslen's test, or thigh thrust). • A maximum of 4 sacroiliac joint injections may be performed in a 12-month period **EXCLUSIONS** These requests are excluded from consideration under this guideline: • Sacral lateral branch blocks (S1, S2, S3) • Radiofrequency denervation of the sacroiliac joint **CONTRAINDICATIONS [1, 3]** • Active systemic or spinal infection • Skin infection at the site of needle puncture • Local malignancy	least 50% pain relief or significant documented functional improvement is obtained *Therapeutic Phase* • Sacroiliac joint injections may only be repeated after the initial treatment phase if the individual has had at least 50% pain relief or significant documented functional improvement for a **minimum of 2 months** before each therapeutic injection • The individual continues to have pain causing functional disability or average pain level ≥ 6 on a scale of 0 to 10 related to the requested spinal region. • The individual is engaged in ongoing active conservative treatment unless the medical reason this treatment cannot be done is clearly documented • For individuals that have received other interventional pain injections in the lumbar/sacral region (e.g., epidural steroid injection or facet joint injection) since the last SIJ injection, at least one repeat positive provocative exam finding is required (pelvic (SI) distraction test, pelvic (SI) compression test, sacral thrust test, FABER (Patrick's test), posterior shear test, Yeoman's test, Gaenslen's test, or thigh thrust). • A maximum of 4 sacroiliac joint injections may be performed in a 12-month period per region in the therapeutic phase **EXCLUSIONS** These requests are excluded from consideration under this guideline: • Sacral lateral branch blocks (S1, S2, S3) • Radiofrequency denervation of the sacroiliac joint **CONTRAINDICATIONS [2,3,4]** • Absolute contraindications ○ Active systemic or spinal infection ○ Skin infection at the site of needle puncture ○ Local malignancy ○ Septic joint

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	• Relative contraindications ○ Coagulopathy ○ Pregnancy ○ Uncontrollable Diabetes ○ Current and uninterrupted use of blood-thinning medication

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